
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the smallest first step.

PREPARED FOR — TYPE C PRACTICE

Practices with an EHR and a workflow that already works.

Your system works. You have a back room of paper and no analytics. That is the whole gap.

This document is written specifically for a practice in your situation. It is not a generic brochure — the recommendation in it is the one configuration we would actually put in your practice.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

Your practice is already digital and your workflow already hangs together — so we are not going to pretend you need a new platform. The demo was deliberately narrow: your legacy archive becoming structured data, and the analytics view your current system does not give you. Those two things. Nothing else.

— Your archive, structured, into the system you already run

We took your own legacy files, extracted them into structured records, and showed the export landing in the format your EHR ingests. No change to your system of record.

— The analysis your EHR was never built for

We asked a natural-language question across the archive — the cross-practice view your current system stores the data for but cannot surface — and it answered with the source attached.

— Nothing about your working setup disturbed

No platform migration, no workflow change, no retraining. A layer on top of what already works.

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

<i>Per document</i>	30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.
<i>Week one</i>	Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.
<i>Month three</i>	The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.
<i>What does not change</i>	How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

03 WHAT HAPPENS WHEN IT DOESN'T WORK PERFECTLY

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

You do not need the practice platform and we are not going to sell it to you. Your gap is precisely two things: a back room that is not structured data, and no analytical view of the data you already hold. We address exactly those and nothing else.

Digitisation of the back room

Your legacy archive becomes structured patient data, exported into the EHR you already run via FHIR, CSV or JSON. Your system of record does not move.

The analytics view

Chronic-disease cohorts, claims aging, no-show and productivity patterns, semantic search across the whole archive — ingested from your existing EHR via API or scheduled export. The layer your current system was never meant to be.

No platform change. Deliberately.

We are not quoting you a platform tier. You did not ask for one and you do not need one. The recommendation is two modules, on top of what you already run.

If anything in this document reads like a platform upsell, we have written it wrong — tell us. A Type C practice buys digitisation and analytics, or it buys nothing. We would rather sell you the right small thing than the wrong large one.

The same discipline we apply to the product, applied to this document.
These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in the configuration above, treat it as not yet real.

THE SMALLEST FIRST STEP

One month. Your files. No lock-in.

One month, 1,500 pages of your choosing, exported in the exact format your EHR ingests. The single thing you are testing is whether the extraction quality on *your* documents is good enough and whether the export lands cleanly in the system you already run. If both are true, we talk about the rest of the archive. If either is not, you have lost a month's subscription and nothing else. No setup fee, no lock-in.

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